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A guide to taking a patient's history

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Summary

This article outlines the process of taking a history from a patient, including preparing the environment, communication skills and the importance of order. The rationale for taking a comprehensive history is also explained.

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TAKING A PATIENT history is arguably the most important aspect of patient assessment, and is increasingly being undertaken by nurses (Crumbie 2006). The procedure allows patients to present their account of the problem and provides essential information for the practitioner.

Nurses are continually expanding their roles, and with this their assessment skills. It is likely that history taking will be performed by a nurse practitioner or specialist nurse, although it can be adapted to most nursing assessments. The history is only one part of patient assessment and is likely to be undertaken in conjunction with other information gathering techniques, such as the single assessment process, and nursing assessment.

History taking for assessment of healthcare needs is not new. Many nursing theorists have examined health deficits (Henderson 1966, Roper *et al* 1990, Orem 1995), all of which rely on careful assessment of patients' needs. Other nursing theorists identified interaction theories (Peplau 1952, Orlando 1961, King 1981), which sought to develop the relationship between the patient and the nurse through systematic assessment of health.

This article provides the reader with a framework in which to take a full and comprehensive history from a patient.

Preparing the environment

The first part of any history-taking process and, indeed, most interactions with patients is preparation of the environment. Nurses can encounter patients in a variety of environments: accident and emergency; general wards; department areas; primary care centres; health centre clinics and the patient's home. It is important that the environment in practical terms is accessible, appropriately equipped, free from distractions and safe for the patient and the nurse (Crouch and Meurier 2005).

Respect for the patient as an individual is an important feature of assessment, and this includes consideration of beliefs and values and the ability to remain non-judgemental and professional (Rogers 1951). Respect also involves maintenance of privacy and dignity; the environment should be private, quiet and ideally, there should be no interruptions. When this is not possible the nurse should do everything possible to ensure that patient confidentiality is maintained (Crouch and Meurier 2005).

It is essential to allow sufficient time to complete the history. Not allowing enough time can result in incomplete information, which may adversely affect the patient's care.

Communication

The importance of taking a comprehensive history cannot be overestimated (Crumbie 2006). The nurse should be able to gather information in a systematic, sensitive and professional manner. Good communication skills are essential.

Introducing yourself to the patient is the first part of this process. It is important to let patients tell their story in their own words while using active listening skills. It is also important not to appear rushed, as this may interfere with the patient's desire to disclose information (Hurley 2005). Developing a rapport with the patient includes being professionally friendly, showing interest and actively using both non-verbal and verbal communication skills (Mehrabian 1981) (Box 1).

Practitioners should avoid the use of technical terms or jargon and, whenever possible, use the patient's own words.

BOX 1

Examples of non-verbal and verbal communication skills

Non-verbal	Verbal
Eye contact	Appropriate language
Interested posture	Avoid jargon and technical terms
Nodding of head	Pitch
Hand gestures	Rate and intonation
Clothing	Volume
Facial gestures	

(Mehrabian 1981)

Consent

Before any healthcare intervention, including history taking, informed consent should be gained from the patient. It can be obtained using various methods. However, both the Nursing and Midwifery Council's (NMC 2004) *Code of Professional Conduct* and the Department of Health's (DH 2001) *Good Practice in Consent Implementation Guide* state that patients can only provide consent if they are able to act under their own free will, have an understanding of what they have agreed to and have enough information on which to base a decision.

The ability of the patient to give consent to history taking is important. Consent is governed by two acts of parliament: the Mental Capacity Act 2005 in England and Wales and the Adults with Incapacity (Scotland) Act 2000 in Scotland. There is currently no equivalent law on mental capacity in Northern Ireland. In addition, each health trust will have a local policy that the nurse should follow. The NMC (2007a) and DH (2007a) websites provide further information on the Mental Capacity Act 2005 and consent.

The history-taking process

There are some general principles to follow when gathering information from patients.

Introductions As stated earlier, always begin with preparing the environment, introducing yourself, stating your purpose and gaining consent. Once this has been completed, it is best to begin by establishing the identity of the patient and how he or she would like to be addressed (Hurley 2005). The first information to be gathered as with any history is basic demographic details, such as name, age and occupation.

Order and structure The general structure of history taking follows the process outlined in Box 2. There is a consensus in medical and nursing texts that it is important to have a logical and systematic approach (Douglas *et al* 2005, Crumbie 2006). Many books and articles also suggest that the history should be taken in a set order (Douglas *et al* 2005, Shah 2005), however, it is not necessary to adhere to these rigidly.

Open questions It is important to use appropriate questioning techniques to ensure that nothing is missed when taking a history from a patient. Always start with open-ended questions and take time to listen to the patient's story. This can provide a great deal of information, although not necessarily in a systematic order. Examples of open questioning include: 'Tell me about your health problems?' and 'How does this affect you?'

Closed questions Once the patient has completed his or her 'story' move on to clarify and focus with specific questions. Closed questions provide extra detail and sharpen the patient's story. Examples of closed questioning include: 'When did it begin?' and 'How long have you had it for?'

Clarification Clarification involves recalling back to the patient your understanding of the history, symptoms and remarks. Summarising the history back to the patient is necessary to check that you have got it right and to clarify any discrepancies. Finally, asking the patient, 'Is there anything else?' gives him or her a final opportunity to add any further information.

In general, interviewing skills develop through practice. Some helpful points of guidance to consider include (Morton 1993):

- Encouraging participation and agreement.
- Offering prompts and general leads.
- Focusing the discussion.
- Placing symptoms or problems in sequence.
- Using pauses effectively.
- Making observations that encourage the patient to discuss symptoms.
- Reflecting.

BOX 2

History-taking sequence

- The presenting complaint.
- Past medical history.
- Mental health.
- Medication history.
- Family history.
- Social history.
- Sexual history.
- Occupational history.
- Systemic enquiry.
- Further information from a third party.
- Summary.

(Adapted from Douglas *et al* 2005)

- Clarifying points by restating points raised.
- Summarising.

There are also some techniques that should be avoided. These are outlined by Crumbie (2006) (Box 3).

Calgary Cambridge framework

Kurtz *et al* (2003) refined the Calgary Cambridge Observation Guide (CCOG) model of consultation to include structuring the consultation. The CCOG is useful as it facilitates continued learning and refining of consultation skills for the teacher and practitioner and is an ideal model for both 'novice' and 'experienced' nurses. Kurtz *et al* (2003) suggested five stages to summarise history taking including:

Explanation and planning Giving patients information, checking that it is correct and that you both agree with the history that has been taken.

Aiding accurate recall and understanding Making information easier for the patient using reflection.

Achieving a shared understanding Incorporating the patient's perspective to encourage an interaction rather than a one-way transmission.

Planning through shared decision making Working with patients to assist understanding and involving patients in the decision-making process.

Closing the consultation Explaining, checking and offering a plan acceptable to the patient's needs and expectations.

BOX 3

Examples of unhelpful interview techniques

- Asking 'why' or 'how' questions.
- Using probing persistent questions.
- Using inappropriate or technical language.
- Giving advice.
- Giving false reassurance.
- Changing the subject or interrupting.
- Using stereotype responses.
- Giving excessive approval or agreement.
- Jumping to conclusions.
- Using defensive responses.
- Asking leading questions that suggest right answers.
- Social chat: the person is expecting professional expertise.

(Crumbie 2006)

Taking the history

If the structure advised by Douglas *et al* (2005) is used, history taking should start with asking the patient about the presenting complaint.

The presenting complaint To elicit information about the presenting complaint start by using an open question, for example: 'What is the problem?' or 'Tell me about the problem?'. This should provide a breadth of valuable information from the patient, but not necessarily in the order that you would like. The patient should then be asked more specific details about his or her symptoms, starting with the most important first. It is important to concentrate on symptoms and not on diagnosis to ensure that no information is missed. Most textbooks provide a list of cardinal symptoms – those symptoms that are most important to that body system – and should be asked about to ensure that a full history is obtained from the patient. Box 4 provides a list of examples of the cardinal symptoms for each body system.

When a patient reports symptoms from a specific body system, all of the cardinal symptoms in the system should be explored. For example, if a patient complains of palpitations, then specific questions should be asked about chest pain, breathlessness, ankle swelling and pain in the lower legs when walking to ensure that all cardinal questions relating to the cardiovascular system have been covered.

Each symptom should be explored in more detail for clarification because this helps to construct a more accurate description of the patient's problems. Direct questions can be used to ask about:

- Onset – was it sudden, or has it developed gradually?
- Duration – how long does it last, such as minutes, days or weeks?
- Site and radiation – where does it occur? Does it occur anywhere else?
- Aggravating and relieving features – is there anything that makes it better or worse?
- Associated symptoms – when this happens, does anything else happen with it, such as nausea, vomiting or headache?
- Fluctuating – is it always the same?
- Frequency – have you had it before?

Direct questioning can be used to ask about the sequence of events, how things are currently and any other symptoms that might be associated with possible differential diagnoses and risk factors. Negative responses are also important, and it is vital to understand how the symptoms affect the patient's day-to-day activities.

Past medical history When a full account of the presenting complaint has been ascertained, information about the patient's past medical history should be gathered. This may provide essential background information – for example, on diabetes and hypertension, or a past history of cancer. It is important to capture the following information when taking a past medical history:

- Diagnosis.
- Dates.
- Sequence.
- Management.

Begin by using questions such as, 'What illnesses have you had?' Ensure that you have obtained a full list of the patient's past medical history and explore each of these in detail as with the presenting complaint. It is useful to prompt the patient by using direct questioning to ask about common major medical illnesses, such as whether he or she has ever had tuberculosis; rheumatic fever; heart disease; hypertension; stroke; diabetes; asthma; chronic obstructive pulmonary disease; or epilepsy.

Mental health According to the NHS Confederation (2007), one in four people will experience mental health problems at one time during their life. This figure demonstrates that nurses are likely to encounter mental health issues frequently. By using skills previously highlighted, and with a supportive and professional approach, the nurse can enquire with confidence about the patient's current coping strategies, such as anxieties over health problems (suspicion of malignancy, impending surgery or test results) or more developed mental health issues, such as bipolar disorder or schizophrenia.

Further clues can be gained from the patient's prescribed medication history or previous hospital admissions. The nurse may feel anxious about enquiring about mental health issues, but it is an important part of wellbeing and should be assessed.

Medication history This is crucially important and should consider not only what medication the patient is currently taking but also what he or she might have been taking until recently. Because of the availability of so many medications without prescription, known as over-the-counter drugs, remember to ask specifically about any medications that have been bought at the pharmacy or supermarket, including homeopathic and herbal remedies. For each medication ask about: the generic name, if possible; dose; route of administration; and any recent changes, such as increase or decrease in dose or change in the amount of times the patient takes the medication.

BOX 4

Cardinal symptoms

General health
- Wellbeing
- Energy
- Appetite
- Sleep
- Weight change
- Mood/anxiety/stress

Cardiovascular system
- Chest pain
- Breathlessness
- Palpitations
- Ankle swelling
- Pain in lower leg when walking

Central nervous system
- Headaches
- Dizziness
- Vertigo
- Sensations
- Fits/faints
- Weakness
- Twitches
- Tinnitus
- Visual disturbance
- Memory and concentration changes

Endocrine
- Excessive thirst
- Tiredness
- Heat intolerance
- Hair distribution
- Change in appearance of eyes

Gastrointestinal system
- Dental/gum problems
- Tongue
- Difficulty in swallowing
- Painful swallowing
- Nausea
- Vomiting
- Heartburn
- Colic
- Abdominal pain
- Change in bowel habit
- Colour of stools

Genitourinary system
- Pain on urinating
- Blood in urine
- Risk assessment for sexually transmitted infections

Men
- Hesitancy passing urine
- Frequency of micturition
- Poor urine flow
- Incontinence
- Urethral discharge
- Erectile dysfunction
- Change in libido

Musculoskeletal
- Joint pain
- Joint stiffness
- Mobility
- Gait
- Falls
- Time of day pain

Respiratory system
- Shortness of breath
- Cough
- Wheeze
- Sputum
- Blood in sputum
- Pain when breathing

Women
- Onset of menstruation
- Last menstrual period
- Timing and regularity of periods
- Length of periods
- Type of flow
- Vaginal discharge
- Incontinence
- Pain during sexual intercourse

(Adapted from Douglas *et al* 2005)

Concordance with medication is an important part of taking a medication history. Finding out the level of concordance and any reasons for non-concordance can be of significance in the future treatment of the patient. Finally, ask about any allergies and sensitivities, especially drug allergies, such as allergy or sensitivity to penicillin. It is important to find out what the patient experienced, how it presented in terms of symptoms, when it occurred and whether it was diagnosed.

Family history Some disorders are considered familial; a family history can reveal a strong history of, for example, cerebrovascular disease or a history of dementia, that might help to guide the management of the patient. Open questioning followed by closed questioning can be used to gather information about any significance in the patient's family history. For example, start with an open question such as: 'Are there any illnesses in the family?' Then ask specifically about immediate family – namely parents and siblings. For each individual ask about diagnosis and age of onset and, if appropriate, age and cause of death.

Social history A patient's ability to cope with a change in health depends on his or her social wellbeing. A level of daily function should be established throughout the history taking. The nurse should be mindful of this level of function and any transient or permanent change in function as a result of past or current illness.

Questions about function should include the ability to work or engage in leisure activities if retired; perform household chores, such as housework and shopping; perform personal requirements, such as dressing, bathing and cooking. In particular, with deteriorating health a patient may have needed to give up club or society memberships, which may lead to a sense of isolation or loss.

Nurses should consider the whole of the family when exploring a social history. Relationships to the patient should be explored, for example, is the patient married, is his or her spouse healthy, do they have children and, if so, what age are they? The health and residence to the patient should be known to understand actual and potential support networks. Other support structures include asking about friends and social networks, including any involvement of social services or support from charities, such as MIND (National Association for Mental Health) or the Stroke Association.

The social history should also include enquiry into the type of housing in which the patient lives. This should include if the accommodation is owned, rented or leased, what condition it is in and whether there have been any adaptations.

Alcohol In relation to the social history ask specifically about alcohol intake. The nurse should ask about past and present patterns of drinking alcohol. Ewing (1984) suggested use of the CAGE system, in which four questions may elicit a view of alcohol intake (Box 5). Hearne *et al* (2002) considered it to be an efficient screening tool.

The nurse should be wary of patients who are evasive or indignant when asked questions about alcohol consumption. A mental note should be taken to ask again at a later stage and to consider physical evidence of alcohol intake during the physical examination. Many patients do not recognise units of alcohol and will talk in measures and volume for which the nurse will have to have a mental ready reckoner to calculate the weekly alcohol consumption. The DH website provides useful guidance on this (Box 6).

BOX 5

The CAGE system

- Have you ever felt the need to **C**ut down?
- Have people **A**nnoyed you by criticising your drinking?
- Have you ever felt **G**uilty about your drinking?
- Have you ever had a drink to steady your nerves in the morning (**E**ye opener)?

(Ewing 1984)

BOX 6

Equivalent units of alcohol

- A pint of ordinary strength lager, for example, Carling Black Label, Foster's = 2 units.
- A pint of strong lager, for example, Stella Artois, Kronenbourg 1664 = 3 units.
- A pint of ordinary bitter, for example, John Smith's, Boddingtons = 2 units.
- A pint of best bitter, for example, Fuller's ESB, Young's Special = 3 units.
- A pint of ordinary strength cider, for example, Woodpecker = 2 units.
- A pint of strong cider, for example, Dry Blackthorn, Strongbow = 3 units.
- A 175ml glass of red or white wine is around 2 units.
- A pub measure of spirits = 1 unit.
- An alcopop, for example, Smirnoff Ice, Bacardi Breezer, WKD, Reef is around 1.5 units.

(DH 2007b)

Nurses should be mindful that increased alcohol consumption might be a reaction to the health stressors affecting the patient during adjustment to recent changes in health. It could also be that the patient is drinking excessively to act as both a physical and emotional analgesic. Careful, but purposeful, questioning using a mixture of the skills outlined should encourage the nurse to have confidence to broach the topic of alcohol dependence. Specific questioning should include the quantity and type of alcohol consumed and where the majority of the drinking takes place, whether in isolation or company.

Smoking It is documented that smoking causes early death in the population and no safe maximum or minimum limit, unlike alcohol, has been identified. Nurses should ask questions that identify the history of the patient's smoking. Traditionally questions surrounding smoking include: 'What age did you start smoking?', 'What kind of cigarettes do you smoke?', 'How many cigarettes a day do you smoke?', 'Do you use roll ups or filtered?' and 'Are they low or high tar content?'.

Patients will often be unclear about the amount they smoke, but with persistence, 'pack years' – now the standard measure of tobacco consumption – can be calculated (Prignot 1987). Pack years is a calculation to measure the amount a person has smoked over a long period. The pack year number is calculated by multiplying the number of packs of cigarettes smoked per day by the number of years the person has smoked. For example, one pack year is equal to smoking one pack per day for one year, or two packs per day for half a year, and so on.

If an individual smokes three packs per day for 20 years then this would amount to 3 packs per day x 20 years = 60 pack years.

Roll-up cigarettes are more difficult to calculate as these are made by the patient and are not a standard size. Tobacco is usually sold in grams but verbalised in ounces. Approximate tobacco amounts can be calculated (Box 7).

Illicit/recreational drugs In the British Crime Survey, Roe and Man (2006) identified that just under half (45.1%) of all 16-24-year-olds have used one or more illicit drugs in their lifetime, 25.2% have used one or more illicit drugs in the last year and 15.1% in the last month.

BOX 7

Approximate calculation of tobacco

1 ounce = 28.34 grams

2 ounces = 56.69 grams

3 ounces = 85.04 grams

A 'standard' pouch of tobacco is equivalent to 50 grams

Recreational drugs are those that are used regularly and which are a focus of a leisure activity without interrupting the user's abilities and lifestyle (Vose 2000). Drug dependence is when recreational use reaches a level of 'tolerance'. This is the point where or when the use of the drug requires larger more regular usage to acquire the same initial effect.

Professional and appropriate behaviour by the nurse, using careful and tactful questioning, is needed to enable the patient to feel comfortable in disclosing drug use. The nurse may uncover unpleasant or illegal actions by the patient in their pursuit of obtaining drugs or being under the influence of drugs.

Sexual history This can be a difficult subject to broach and it is not always appropriate to take a full sexual history (Douglas *et al* 2005). Where relevant ask questions in an objective manner, but acknowledge the sensitivity of the subject by starting with: 'I hope you don't mind but I need to ask some questions about ...'

In men, questions regarding sexual history can be asked as part of the genitourinary system history and should include any previous urinary tract infections, sexually transmitted infections and treatments provided. In women date of menarche, regularity and character of periods, pregnancies, live deliveries and terminations or other losses should be recorded. Women should also be sensitively asked about any infections and treatments. High-risk sexual activity, such as unprotected sexual intercourse should be addressed in both genders. In men and women an enquiry should be made regarding libido, increased or diminished, to reflect both psychological and endocrine systems.

Occupational history Taking a history should include information on previous and current employment. This is important as aspects of employment other than the job itself can influence social wellbeing if illness precludes a return to work. For example, employment in heavy industry may lead to respiratory problems or joint problems. Although occupations may date back several years, exposure to some products may have a long incubation period, such as resultant mesothelioma after asbestos exposure.

Past and current employment will also provide details of financial stability of the home. Retired patients may have financial limitations, as will patients who are currently unemployed. Increased anxiety can be present in patients who find themselves unable to work because of sudden illness or having to care for a relative or partner. Questions about a patient's financial condition should be unhurried and handled sensitively by the nurse. This might include discussion about social support and benefits

because hospitalisation can alter the patient's eligibility for benefits.

Systemic enquiry The final part of history taking involves performing a systemic enquiry. This involves asking questions about the other body systems not discussed in the presenting complaint. The purpose of this is to check that no information has been omitted. It involves systematic questioning of symptoms relating to cardiovascular, respiratory, gastrointestinal, genitourinary, locomotor and dermatological aspects and might yield important clues about the cause of the presenting problems. The cardinal symptoms for each system are outlined in Box 4 and questioning should focus on the presence or absence of these symptoms. It is expected at this stage to receive a negative answer to symptoms not already discussed. However, a positive response to any of the questioning should be investigated using the same method as in the presenting complaint.

It is important not to overlook the value of obtaining a collateral history from a friend or relative. If necessary, and with the patient's permission, use the telephone to obtain this information. It might be essential in a patient presenting with an unexplained loss of consciousness or cognitive symptoms. Information from the history is essential in guiding the treatment and management of a patient. Alternatively, the prescribed medication history may be checked with the GP practice if the patient is not able to give a full history.

Conclusion

This article has presented a practical guide to history taking using a systems approach. It considered the key points required in taking a comprehensive history from a patient, including preparing the environment, communication skills and the importance of order. While this article provides the knowledge for taking a history, the best method of achieving skills in history taking is through a validated training course with competency-based assessments.

The history-taking interview should be of a high quality and must be accurately recorded (Crumbie 2006). Nurses should be familiar with the *NMC Code of Professional Conduct* regarding competence, consent and confidentiality (NMC 2004). The novice history taker's records should adhere to the NMC's (2007b) guidance on record keeping **NS**

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